

Access guide — pdac-recurrent-cldn18-2-kras-g12r-tp53 -y220c-p25m

How to access each therapy: trial contacts + manufacturer medical-info

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How a patient or treating team could practically access each intervention in this case's dossier. Trial recruitment contacts and manufacturer medical-information lines are captured for direct outreach. The number in the first column of the summary table is the entry's identifier in the per-intervention sections below — use it to find the deep section quickly. Information ages — each row carries a [Verified](#) date; re-screen before relying on a specific contact or trial slot.

Summary

#	Intervention	Modality	Access status	Regulatory	Recommended first action
1	CLDN18.2 re-stain on the FDA assay (VENTANA CLDN18 43-14A RxDx)	diagnostic assay	Standard of care	—	Send the December 2025 resection block (or 5 unstained slides) to NeoGenomics or Labcorp now; the archival stain is informative even before the recurrence is biopsied.
2	Comprehensive DNA+RNA NGS on the recurrence with codon-level KRAS	diagnostic assay	Standard of care	—	Order a comprehensive DNA+RNA panel on the recurrence cores as soon as the biopsy is done; confirm the requisition asks for codon-level KRAS, TP53, and fusion calling.
3	Dedicated germline multigene panel (BRCA1/2, PALB2, ATM); BRACAnalysis CDx is the olaparib companion diagnostic	diagnostic assay	Standard of care	—	Order the germline panel now; it is a blood or saliva test and does not depend on the biopsy.
4	EUS-guided core biopsy of the recurrent pancreatic body mass	diagnostic procedure	Standard of care	—	Book EUS with fine-needle biopsy (FNB, core needle) at a high-volume pancreaticobiliary center; flag the vascular involvement on the referral so the endosonographer plans the approach.
5	NALIRIFOX (liposomal irinotecan + oxaliplatin + 5-FU/LV)	chemotherapy	Standard of care	FDA-approved February 2024 (1L metastatic PDAC, NAPOLI-3)	Only revisit if the treating team wants a platinum re-challenge (for example on a positive germline result) and the neuropathy grade permits oxaliplatin.
6	Oxaliplatin neuropathy grading and baseline organ-function labs	clinical assessment	Standard of care	—	Grade the neuropathy formally (CTCAE v5) at the next visit and put the grade in the chart where trial screeners will find it.

7	ctDNA comprehensive liquid panel (Guardant360 CDx or FoundationOne Liquid CDx)	diagnostic assay	Standard of care	—	Order the liquid panel at the next clinic visit; it does not need to wait for the EUS.
8	gemcitabine + nab-paclitaxel after FOLFIRINOX failure	chemotherapy	Standard of care	FDA-approved combination (1L metastatic PDAC); guideline-listed as subsequent therapy after FOLFIRINOX	No procurement steps; if chosen, grade the neuropathy first since nab-paclitaxel adds neurotoxicity.
9	liposomal irinotecan (Onivyde) + 5-FU/leucovorin	chemotherapy	Off-label use	FDA-approved (mPDAC after gemcitabine-based therapy); this case's sequence is outside the labeled population	Hold this in reserve for a post-gemcitabine line, where it becomes on-label, rather than spending it now.
10	olaparib maintenance (gated on the pending germline panel)	small molecule (PARP inhibitor)	Off-label use	FDA-approved (1L maintenance, germline BRCA-mutated metastatic PDAC, POLO); label fit for this case depends on pending germline result and future platinum response	Wait for the germline panel; nothing to do before it returns.
11	pembrolizumab (anti-PD-1)	monoclonal antibody	Off-label use	FDA-approved (multiple indications; tumor-agnostic MSI-H and TMB-H labels); no MSS-PDAC indication	No single-agent action; any PD-1 exposure in this case should come inside a combination trial that provides its own rationale.
12	zolbetuximab (anti-CLDN18.2 mAb) + chemotherapy	monoclonal antibody	Off-label use	FDA-approved October 2024 and EMA-approved for CLDN18.2-positive gastric/GEJ adenocarcinoma; no PDAC indication	Run the 43-14A CDx re-stain first; if the CDx read comes back at or above 75% the off-label conversation changes materially.
13	AN9025 (pan-RAS(ON) inhibitor)	small molecule	Clinical trial	Investigational; no approval	Email about current dose level and G12R slots.

14	ASP2138 (CLDN18.2xCD3 2+1 bispecific T-cell engager)	bispecific T-cell engager	Clinical trial	Investigational; no approval	Call the Astellas trial line (800-888-7704, to ask whether the PDAC expansion is enrolling and whether archival CLDN18.2 tissue can start screening before the biopsy.
15	ASP546C (CLDN18.2 ADC)	ADC	Clinical trial	Investigational; no approval	Ask the Astellas trial line (800-888-7704) whether PDAC is enrolling and at which US sites.
16	ASP5834 (IV pan-KRAS inhibitor)	small molecule	Clinical trial	Investigational; no approval	Call the Astellas trial line (800-888-7704, to ask which US sites are enrolling and whether the PDAC expansion is open at the current dose level.
17	AZD0901 / CMG901 (CLDN18.2 ADC, MMAE payload)	ADC	Clinical trial	Investigational; no approval	One call to the AstraZeneca information center to confirm the 6-month clock is measured to the last adjuvant dose and whether any later-line PDAC cohort is planned.
18	AZD4360 (CLDN18.2 ADC)	ADC	Clinical trial	Investigational; no approval	Registry watch; ask the AstraZeneca information center whether expansion cohorts will reopen enrollment.
19	AZD5863 (CLDN18.2xCD3 affinity-modulated T-cell engager)	bispecific T-cell engager	Clinical trial	Investigational; no approval	Call 1-877-240-9479 with the treatment timeline and ask whether the prior-line requirement is satisfied.
20	BBO-11818 (pan-KRAS inhibitor)	small molecule	Clinical trial	Investigational; no approval	One email to the sponsor contact asking whether G12R has been or will be added; otherwise no action.
21	BLU-924 (SAR449336, pan-KRAS inhibitor)	small molecule	Clinical trial	Investigational; no approval	Email to confirm codon coverage and site openings.
22	ELI-002 mKRAS amphiphile vaccine; 2P and 7P formulations)	vaccine	Clinical trial	Investigational; no approval	Watch for an Elicio trial in the overt-recurrence or metastatic setting; none is posted today.
23	IBI343 (CLDN18.2 ADC, exatecan payload)	ADC	Clinical trial	Investigational; no approval	Email the NCT05458219 contact asking which US sites are enrolling the PDAC cohort and what CLDN18.2 expression threshold applies at 60% 2+/3+.

24	JAB-23E73 (oral pan-KRAS inhibitor)	small molecule	Clinical trial	Investigational; no approval	Email to confirm G12R fits and which US sites have slots.
25	JAB-30355 (oral p53 Y220C reactivator)	small molecule	Clinical trial	Investigational; no approval	Email with the Y220C documentation and treatment history; ask about US site slots and whether KRAS co-mutation affects cohort assignment.
26	KST-6051 (oral pan-KRAS inhibitor)	small molecule	Clinical trial	Investigational; no approval	Email to confirm G12R enrollment and site openings.
27	LB1908 (CLDN18.2 CAR-T)	CAR-T	Clinical trial	Investigational; no approval	Watch NCT05539430 for reopening or a phase 2 posting.
28	LG00313112 (oral p53 Y220C reactivator)	small molecule	Clinical trial	Investigational; no approval	Set a registry alert on NCT07752875 and re-check monthly.
29	Mutant-KRAS TCR-T programs (NT-112/AZD0240, Anocca ANOC-001/2/3, TCR1188)	TCR-T	Clinical trial	Investigational; no approval	One email to clinical.trials@anocca.com asking whether a G12R-restricted TCR is in their library plans; their platform is built to broaden allele coverage.
30	NTS071 (oral p53 Y220C reactivator)	small molecule	Clinical trial	Investigational; no approval	Email the posted contact with Y220C documentation and ask which US site is enrolling and at what dose level.
31	PF-07934040 (pan-KRAS inhibitor)	small molecule	Clinical trial	Investigational; no approval	Watch NCT06447662 for a status change; no live path today.
32	QLS31905 / ANO31905 (CLDN18.2xCD3 bispecific) +	bispecific T-cell engager	Clinical trial	Investigational; no approval	Email the posted investigator contact (zhengl@uthscsa.edu, 210-450-5798) with the case summary and CLDN18.2 report.
33	TAC01-CLDN18.2 (T-cell antigen coupler cell therapy)	engineered T cell	Clinical trial	Investigational; no approval	Watch for reopening; screen the recruiting cell-therapy alternatives meanwhile.
34	TG01 mutant-KRAS peptide vaccine + daratumumab + nivolumab (Georgetown)	vaccine	Clinical trial	Investigational; no approval	No action; the Hopkins mKRASvax study is the recruiting alternative in this class.

35	TNhYP218 (mesothelin-targeted CAR-T, NCI)	CAR-T	Clinical trial	Investigational; no approval	Email the posted referral contact (mariagracia.agra@nih.gov , (240) 858-3152) with records; NCI screening will handle mesothelin testing.
36	TORL-2-307-ADC (CLDN18.2 ADC)	ADC	Clinical trial	Investigational; no approval	Watch NCT05156866 for expansion-phase reopening.
37	YL-17231 (pan-RAS inhibitor)	small molecule	Clinical trial	Investigational; no approval	Contact the posted trial contact (David S. Hong, MD, MD Anderson) about slots and dose level.
38	daraxonrasib (RMC-6236, pan-RAS(ON) multi-selective inhibitor)	small molecule	Clinical trial + Expanded access program	Investigational; NDA accepted for FDA review July 2026 (previously treated metastatic PDAC), Breakthrough Therapy and Orphan Drug designations, FDA national priority voucher; FDA-authorized expanded access protocol active since May 2026	Before the biopsy result returns, have the oncologist identify the nearest RASolute 303 site (site list on NCT07491445) and pre-alert them, so consent can happen inside the 6-week window if staging reads metastatic.
39	givastomig (TJ033721/ABL111, CLDN18.2x4-1BB bispecific)	bispecific antibody	Clinical trial	Investigational; no approval	Email us.info@imabbio.com asking whether the PDAC combination cohort is open at a US site and what expression cutoff applies.
40	mKRASvax pooled mutant-KRAS peptide vaccine + balstilimab + botensilimab (Johns Hopkins)	vaccine	Clinical trial	Investigational; no approval	Email GIClinicalTrials@jhmi.edu with the treatment timeline and ask whether perioperative FOLFIRINOX with early recurrence satisfies the cohort's prior-therapy requirement.
41	rezatapopt (PC14586, p53 Y220C reactivator)	small molecule	Clinical trial + Compassionate use	Investigational; no approval (PYNNACLE registrational program ongoing)	Confirm Y220C persists in the recurrence (ctDNA suffices) before spending calls on this class.
42	sacituzumab tirumotecan (sac-TMT, MK-2870/SKB264, TROP2 ADC)	ADC	Clinical trial	NMPA-approved (2L+ TNBC, China); investigational in the US, FDA breakthrough designation in EGFR-mutant NSCLC	Call 1-888-577-8839 or email Trialsites@msd.com to locate the nearest US site with PDAC-arm slots and confirm the prior-therapy definition.

43	spevatamig (PT886, CLDN18.2xCD47 bispecific)	bispecific antibody	Clinical trial	Investigational; no approval (FDA orphan and fast track designations reported for pancreatic cancer)	Email clinical-trials@phanestx.com for site openings and whether archival tissue can pre-screen while the biopsy is scheduled.
44	zoldonrasib (RMC-9805, KRAS G12D(ON)-selective inhibitor)	small molecule	Clinical trial	Investigational; no approval	No action; the allele does not match. Daraxonrasib is the Revolution Medicines route for G12R.
45	(CLDN18.2-directed lutetium-177 radioligand therapy)	radioligand	Not yet accessible	Investigational; no approval anywhere, no US IND posted	One email to mengru.wu@smartnucl.com from the treating team, asking two questions: does the IIT have any mechanism for a non-resident patient, and is a US or multinational sponsor study planned. Expect the first answer to be no; the second is the useful one.
46	AZD6422 (TGF-beta-armored CLDN18.2 CAR-T; class exemplar for stromal suppression)	CAR-T	Not yet accessible	Investigational; no approval	Ask the AstraZeneca information center whether a global AZD6422 study is planned; otherwise no action.
47	FG-M108 (ADCC-enhanced anti-CLDN18.2 mAb) +	monoclonal antibody	Not yet accessible	Investigational; no approval	Email pr@futuregenbiopharm.com asking whether the phase 3 will open ex-China sites or whether any global study is planned.
48	GenSci128 (p53 Y220C reactivator)	small molecule	Not yet accessible	Investigational; no approval	No US action; revisit if a US site appears on the registry.
49	JYP0015 (ERAS-0015, pan-RAS(ON) molecular glue)	small molecule	Not yet accessible	Investigational; no approval	No US action; revisit if Erasca or the sponsor opens a US IND for this molecule.
50	XNW27011 (CLDN18.2 ADC, licensed by Astellas from Evopoint)	ADC	Not yet accessible	Investigational; no approval	Watch for an Astellas global study posting; ask their medical line if one is imminent.
51	[68Ga]/[131I]-IR199 (CLDN18.2-directed theranostic pair, single dose)	radioligand	Not yet accessible	Investigational; no approval anywhere, no US IND posted	No outreach worth the patient's time; a single-dose dosimetry study in Shanghai offers neither access nor treatment intent for a US patient.

52	satri-cel (CT041, autologous CLDN18.2 CAR-T)	CAR-T	Not yet accessible	NMPA-approved June 2026 (CLDN18.2+ gastric/GEJ, 2L+); investigational in the US, no FDA filing announced	Have the treating team email PR@cargen.com asking about US reopening, expanded-access plans, or any named-patient route for pancreatic cancer.
53	EO-3021 / SYSA1801 (CLDN18.2 ADC, MMAE payload)	ADC	Unavailable	Discontinued (Elevation Oncology, March 2025); no approval	None; the recruiting CLDN18.2 ADC alternatives are IBI343 and ASP546C.

Standard of care (8)

1. CLDN18.2 re-stain on the FDA companion-diagnostic assay (VENTANA CLDN18 43-14A RxDx)

Modality: diagnostic assay · **Geographic scope:** US send-out from any pathology department · **Verified:** 2026-08-12

The FDA-approved 43-14A CDx assay is available as a send-out from at least three US reference labs, so re-staining the archival 2025 resection block can happen this week without waiting for a biopsy. NeoGenomics runs it in 1 to 2 days, Labcorp Oncology (test 452390) in about 3, and Mayo Clinic Laboratories lists it as test CLD18. We could not confirm that Tempus or Caris offer the 43-14A CDx assay, which matches what the target validator found, so route the order to one of the three verified labs. The prior 60% 2+/3+ call was on an unstated clone; the CDx re-stain settles where this tumor really sits against the 75% zolbetuximab cutoff and the lower trial cutoffs (FG-M108's phase 3 uses 40% or more at 2+ or higher).

Next steps

1. Send the December 2025 resection block (or 5 unstained slides) to NeoGenomics or Labcorp now; the archival stain is informative even before the recurrence is biopsied.
2. When recurrence cores exist, repeat the stain on current tissue, since CAR-T and ADC sponsors often require it and expression can shift after chemotherapy.
3. File both reports with the percentage at 2+/3+ stated explicitly, so each CLDN18.2 program's cutoff can be checked without re-review.

Manufacturer / sponsor contact

- **Company:** NeoGenomics (test 'Claudin 18 FDA (VYLOY)'); Labcorp Oncology (test 452390); Mayo Clinic Laboratories (test CLD18 / 620665)
- **Med info phone:** 866.776.5907
- **Product info:**
<https://www.neogenomics.com/providers/test/APH-CLAP-01AX/claudin-18-fda-vyloy-for-gastric-gej-adenocarcinoma>
- **Notes:** Phone is NeoGenomics client services (option 3), listed on the test page; TAT 1-2 days, tech-only in 24 hours. Labcorp page: <https://oncology.labcorp.com/tests/452390/claudin-18-ihc> (TAT 3 days). Mayo page: <https://www.mayocliniclabs.com/test-catalog/overview/620665>.

Payer / coverage: IHC on archival tissue is routinely covered as pathology; the FDA CDx billing code is established through the zolbetuximab approval.

Notes: Note the assay reads total CLDN18 (both isoforms) via the C-terminus; in gastric and pancreatic tumors CLDN18.2 is the dominant isoform, which is why the CDx works.

2. Comprehensive DNA+RNA NGS on the recurrence with codon-level KRAS

Modality: diagnostic assay · **Geographic scope:** US, mail-in from any treating center · **Verified:** 2026-08-12

Commercially available from multiple CLIA labs (FoundationOne CDx, Tempus xT/xR, Caris MI Profile) and orderable by any oncologist; all of these report KRAS at codon level, which matters here because G12R gates which RAS programs the patient fits. The constraint is tissue: the recurrence has to be biopsied first, and the lab needs adequate tumor content from the cores.

Next steps

1. Order a comprehensive DNA+RNA panel on the recurrence cores as soon as the biopsy is done; confirm the requisition asks for codon-level KRAS, TP53, and fusion calling.
2. Ask the lab to hold residual tissue rather than exhaust the block, since CLDN18.2 IHC and possible sponsor central labs will want sections too.
3. Have the report sent to whoever runs trial screening so the G12R and Y220C persistence calls flow straight into eligibility packets.

Payer / coverage: Medicare covers FDA-approved comprehensive NGS for advanced or recurrent cancer under NCD 90.2; commercial payers broadly follow. Labs run financial-assistance programs when prior authorization stalls.

3. Dedicated germline multigene panel (BRCA1/2, PALB2, ATM); BRACAnalysis CDx is the olaparib companion diagnostic

Modality: diagnostic assay · **Geographic scope:** US, blood/saliva kit from any clinic · **Verified:** 2026-08-12

Guidelines recommend germline testing for every pancreatic cancer patient, and the incidental tumor-NGS reporting this case currently relies on does not rule those genes out. Myriad's BRACAnalysis CDx is the FDA companion diagnostic tied to the olaparib pancreatic indication and returns results in about 14 days; a broader multigene panel (Invitae, Ambry, Myriad MyRisk) covers PALB2 and ATM as well. The result gates the entire PARP and platinum-emphasis branch of this dossier.

Next steps

1. Order the germline panel now; it is a blood or saliva test and does not depend on the biopsy.
2. If a BRCA1/2 pathogenic variant returns, order or confirm BRACAnalysis CDx specifically, since it is the assay named in the olaparib label.
3. Route any positive to genetic counseling, both for treatment gating and for family implications.

Manufacturer / sponsor contact

- **Company:** Myriad Genetics
- **Med info phone:** 800-469-7423
- **Product info:** <https://myriad.com/genetic-tests/braanalysiscdx-germline-test/>
- **Notes:** Phone is Myriad's billing and coverage line from the BRACAnalysis CDx patient guide. Ordering is via requisition form or the Myriad provider portal.

Payer / coverage: Broadly covered for pancreatic cancer under NCCN-aligned payer policies; Myriad states most appropriate patients pay nothing out of pocket.

4. EUS-guided core biopsy of the recurrent pancreatic body mass

Modality: diagnostic procedure · **Geographic scope:** US, routine at academic and large community centers · **Verified:** 2026-08-12

Routine at any high-volume pancreas center and the single gate in front of almost everything else in this dossier: trial screening, CLDN18.2 re-staining, and re-sequencing all want current tissue. The likely vascular invasion around the mass can complicate the needle path, so this belongs with an experienced pancreaticobiliary endosonographer, and the passes should be planned up front to yield cores for DNA+RNA NGS plus IHC, not just cytology.

Next steps

1. Book EUS with fine-needle biopsy (FNB, core needle) at a high-volume pancreaticobiliary center; flag the vascular involvement on the referral so the endosonographer plans the approach.
2. Specify on the requisition that cores are needed for comprehensive DNA+RNA NGS, CLDN18.2 IHC, and confirmation of PDAC histology; ask the endosonographer to take enough passes for all three.
3. If the mass proves unsafe to biopsy, fall back to the ctDNA liquid panel (separate row) and the archival 2025 resection block for CLDN18.2 re-staining.

Payer / coverage: EUS biopsy of a suspected recurrence is standard diagnostic care and routinely covered; no special authorization pathway applies.

Notes: Biopsy also settles whether the recurrence is treated as locally recurrent or metastatic disease, which changes which trials and the daraxonrasib EAP this patient can reach.

5. NALIRIFOX (liposomal irinotecan + oxaliplatin + 5-FU/LV)

Modality: chemotherapy · **Regulatory:** FDA-approved February 2024 (1L metastatic PDAC, NAPOLI-3) · **Guidelines:** Guideline-listed 1L option for metastatic PDAC · **Geographic scope:** US routine prescription · **Verified:** 2026-08-12

On-label first-line therapy for metastatic PDAC and orderable anywhere, so if restaging reads the recurrence as new metastatic disease this is the approved 1L standard on paper. The dossier's clinical context cuts against it: the regimen overlaps FOLFIRINOX in every component that just failed, and it re-challenges oxaliplatin into ungraded

residual neuropathy. Access easy, rationale weak; the ranking layer owns that tradeoff.

Next steps

1. Only revisit if the treating team wants a platinum re-challenge (for example on a positive germline result) and the neuropathy grade permits oxaliplatin.

Manufacturer / sponsor contact

- **Company:** Ipsen
- **Med info phone:** (844) 441-6225
- **Product info:** <https://us.ipсенmedicalinformation.com/>

Payer / coverage: Covered on-label for 1L metastatic disease; the FOLFIRINOX history will not block authorization but may draw a medical-necessity question.

6. Oxaliplatin neuropathy grading and baseline organ-function labs

Modality: clinical assessment · **Geographic scope:** US, any oncology clinic · **Verified:** 2026-08-12

A clinic-visit item, not a procurement problem: CTCAE grading of the residual neuropathy after 10 FOLFIRINOX cycles plus CBC, CMP, bilirubin, and creatinine clearance. Nearly every protocol in this dossier requires labs within 7 to 28 days of first dose and many cap neuropathy at grade 1, so having current numbers on file is what lets a screening call turn into a screening visit.

Next steps

1. Grade the neuropathy formally (CTCAE v5) at the next visit and put the grade in the chart where trial screeners will find it.
2. Draw baseline CBC, CMP, bilirubin, INR/PTT, and calculate creatinine clearance; repeat within the window a target protocol specifies.
3. Document post-Whipple pancreatic enzyme use and nutritional status, since some protocols ask about absorption for oral agents.

Payer / coverage: Routine evaluation and management; no access barrier.

7. ctDNA comprehensive liquid panel (Guardant360 CDx or FoundationOne Liquid CDx)

Modality: diagnostic assay · **Geographic scope:** US, blood draw at any treating center · **Verified:** 2026-08-12

A blood draw orderable now, before any biopsy, and the route to confirming that KRAS G12R and TP53 Y220C persist in the recurrence if tissue proves hard to get. Several protocols in this dossier accept ctDNA documentation of the target variant, so the same report doubles as trial-entry evidence. The caveat is sensitivity: a small, mostly local recurrence can shed little ctDNA, so a negative result does not exclude persistence.

Next steps

1. Order the liquid panel at the next clinic visit; it does not need to wait for the EUS.
2. If tissue biopsy is deferred or fails, use the ctDNA report as the variant-persistence document for Y220C and G12R trial screening.
3. Repeat or upgrade to tissue NGS once cores exist, since IHC targets like CLDN18.2 cannot be read from blood.

Manufacturer / sponsor contact

- **Company:** Guardant Health (Guardant360 CDx); Foundation Medicine (FoundationOne Liquid CDx)
- **Med info phone:** 855.698.8887
- **Med info email:** clientservices@guardanthealth.com
- **Product info:** <https://www.guardantcomplete.com/hcp/solutions/guardant360-liquid-cdx>
- **Notes:** Phone and email are Guardant client services, captured from the FDA-posted Guardant360 CDx labeling. Foundation Medicine orders go through the treating center's existing Foundation account or foundationmedicine.com.

Payer / coverage: Both FDA-approved liquid panels are covered by Medicare under NCD 90.2 for advanced/recurrent cancer; both companies run patient-assistance billing programs.

8. gemcitabine + nab-paclitaxel after FOLFIRINOX failure

Modality: chemotherapy · **Regulatory:** FDA-approved combination (1L metastatic PDAC); guideline-listed as subsequent therapy after FOLFIRINOX · **Guidelines:** NCCN-listed subsequent-therapy option after fluoropyrimidine-based therapy · **Geographic scope:** US routine prescription · **Verified:** 2026-08-12

The default second-line standard this whole dossier is measured against: both drugs are approved, on formulary everywhere, and startable within days through any oncology practice. The access question is nil; the decision question, cross-resistance after 10 FOLFIRINOX cycles and the residual neuropathy interacting with nab-paclitaxel, belongs to the ranking layer.

Next steps

1. No procurement steps; if chosen, grade the neuropathy first since nab-paclitaxel adds neurotoxicity.
2. Consider pairing the start decision with trial screening timelines so a chemotherapy start does not silently close a first-line-style trial window (several protocols above count any new systemic line).

Payer / coverage: Covered by Medicare and commercial payers as standard chemotherapy; nab-paclitaxel prior authorization is routine and fast.

Off-label use (4)

9. liposomal irinotecan (Onivyde) + 5-FU/leucovorin

Aliases: Onivyde, nal-IRI

Modality: chemotherapy · **Regulatory:** FDA-approved (mPDAC after gemcitabine-based therapy); this case's sequence is outside the labeled population · **Guidelines:** Guideline-listed post-gemcitabine; not the listed option after FOLFIRINOX · **Geographic scope:** US routine prescription · **Verified:** 2026-08-12

Approved and stocked, but the label is for progression after gemcitabine-based therapy, and this patient is gemcitabine-naïve with an irinotecan-containing regimen already behind them, so use here is off-label on the sequence and pharmacologically awkward after FOLFIRINOX. Access is easy; the fit is the problem, and payers sometimes read the label sequence literally.

Next steps

1. Hold this in reserve for a post-gemcitabine line, where it becomes on-label, rather than spending it now.

Manufacturer / sponsor contact

- **Company:** Ipsen
- **Med info phone:** (844) 441-6225
- **Product info:** <https://us.ipсенmedicalinformation.com/>
- **Notes:** IPSEN CARES patient support: 866-435-5677.

Payer / coverage: Expect prior-authorization friction when the chart shows no prior gemcitabine; IPSEN CARES can assist if the team pursues it anyway.

10. olaparib maintenance (gated on the pending germline panel)

Aliases: LYNPARZA

Modality: small molecule (PARP inhibitor) · **Regulatory:** FDA-approved (1L maintenance, germline BRCA-mutated metastatic PDAC, POLO); label fit for this case depends on pending germline result and future platinum response · **Guidelines:** Guideline-listed maintenance option for gBRCAm mPDAC after platinum response · **Geographic scope:** US routine prescription once gated criteria are met · **Verified:** 2026-08-12

Approved and pharmacy-orderable, but doubly gated for this patient: the label requires a germline BRCA mutation, which is unknown until the dedicated panel returns, and it specifies maintenance after 16 or more weeks of first-line platinum without progression, whereas this patient progressed about 4 months after platinum-containing adjuvant therapy. If the panel comes back BRCA- or PALB2-positive, the realistic shape is off-label maintenance after a response to a new platinum-containing line, argued case-by-case with the payer. Captured now so the pathway is ready the day the germline result lands.

Next steps

1. Wait for the germline panel; nothing to do before it returns.
2. If BRCA1/2-positive, document with BRCAAnalysis CDx and plan a platinum-containing line whose response would set up maintenance.
3. If PALB2-positive, expect an off-label prior-auth argument; the treating team should collect the supporting literature before submitting.

Manufacturer / sponsor contact

- **Company:** AstraZeneca
- **Med info phone:** 1-800-236-9933
- **Product info:** <https://medicalinformation.astrazeneca-us.com/home/prescribing-information/lynparza-tablets.html>
- **Notes:** Access 360 (patient access program) handles benefits verification and copay support; med-info line hours Mon-Fri 8-6 ET.

Payer / coverage: On-label use is broadly covered with the BRACAnalysis CDx result on file. Off-label variants (PALB2, post-progression sequencing) need a documented rationale and sometimes appeal; Access 360 assists.

11. pembrolizumab (anti-PD-1)

Aliases: KEYTRUDA, MK-3475

Modality: monoclonal antibody · **Regulatory:** FDA-approved (multiple indications; tumor-agnostic MSI-H and TMB-H labels); no MSS-PDAC indication · **Guidelines:** Guideline-supported only for MSI-H/dMMR or TMB-H disease; off-guideline here · **Geographic scope:** US routine prescription (biomarker-gated) · **Verified:** 2026-08-12

FDA-approved across many indications including tumor-agnostic MSI-high and TMB-high (10 or more mut/Mb) labels, so prescription is mechanically trivial; this tumor qualifies for neither at MSS and 0.83 mut/Mb, and single-agent PD-1 blockade has repeatedly failed in unselected PDAC. The KEYNOTE-158 basket that generated the tumor-agnostic data is closed. Access is not the barrier; biology is.

Next steps

1. No single-agent action; any PD-1 exposure in this case should come inside a combination trial that provides its own rationale.

Trial pathways

NCT	Phase	Status	Eligible	Central contact
NCT02628067		active not recruiting	no	—

Manufacturer / sponsor contact

- **Company:** Merck
- **Med info phone:** 1-800-672-6372
- **Product info:** <https://www.merckaccessprogram-keytruda.com/>
- **Notes:** Merck National Service Center, Mon-Fri 8:00-7:00 ET.

Payer / coverage: Off-label MSS-PDAC use would fail the biomarker-based coverage criteria payers apply to the tumor-agnostic labels.

12. zolbetuximab (anti-CLDN18.2 mAb) + chemotherapy

Aliases: VYLOY, IMAB362, zolbetuximab-clzb

Modality: monoclonal antibody · **Regulatory:** FDA-approved October 2024 and EMA-approved for CLDN18.2-positive gastric/GEJ adenocarcinoma; no PDAC indication · **Guidelines:** On-guideline for gastric/GEJ 1L; off-guideline in PDAC · **Geographic scope:** US and EU prescription (on-label gastric/GEJ); off-label PDAC at prescriber discretion · **Verified:** 2026-08-12

FDA- and EMA-approved for CLDN18.2-positive (75% or more at 2+/3+) HER2-negative gastric/GEJ adenocarcinoma, so a US oncologist can mechanically prescribe it off-label in PDAC today. The evidence and the assay both cut against doing so: the GLEAM phase 2 in PDAC missed its primary OS endpoint (13.7 vs 13.6 months), and this tumor's 60% staining sits below the label-defining cutoff, so expect payer denial and little support from the data. Astellas' early-access program (NCT06048081) is gastric/GEJ-only with US enrollment closed, so it adds nothing here.

Next steps

1. Run the 43-14A CDx re-stain first; if the CDx read comes back at or above 75% the off-label conversation changes materially.
2. If the treating team still wants to pursue it, call Astellas Medical Communications (800-727-7003) for the PDAC data file and any single-patient options.
3. Weigh the CLDN18.2 trial rows (ADCs, engagers, CAR-T) before committing to an off-label antibody with a negative randomized PDAC readout.

Trial pathways

NCT	Phase	Status	Eligible	Central contact
NCT03826163		active not recruiting	no	—
NCT06048081		—	no	—

Manufacturer / sponsor contact

- **Company:** Astellas Pharma
- **Med info phone:**800-727-7003
- **Med info email:**medical.communications@astellas.com
- **Product info:**<https://www.vyloy.com>
- **Notes:** Medical Communications hours Mon-Fri 8:00-4:30 CST; VYLOY Support Solutions (patient access program) is 1-855-272-6609.

Payer / coverage: Off-label PDAC use is outside compendia and below the assay cutoff, so prior authorization is very likely to fail; VYLOY Support Solutions handles on-label access only. Surface this option to the ranking layer with those constraints attached.

Clinical trial (32)

13. AN9025 (pan-RAS(ON) inhibitor)

Modality: small molecule · **Regulatory:** Investigational; no approval · **Geographic scope:** US sites · **Verified:** 2026-08-12

Phase 1 recruiting at three US sites, and its posted criteria are unusually friendly to this exact history: adjuvant therapy with progression within 6 months explicitly counts as prior therapy. RAS mutation can be documented from tissue or cfDNA, so the ctDNA panel would satisfy the paperwork.

Next steps

- 1. Email AN9025S0101@adlainortye.com about current dose level and G12R slots.
- 2. Note the backfill cohorts accept patients who progressed on a prior pan-RAS(ON) inhibitor, a rare post-daraxonrasib option worth remembering.

Trial pathways

NCT	Phase	Status	Eligible	Central contact
NCT07252479		recruiting	likely	Clinical Trial Manager; AN9025S0101@adlainortye.com ; 848 230-7430

Manufacturer / sponsor contact

- **Company:** Adlai Nortye Biopharma
- **Med info phone:** 848 230-7430
- **Med info email:** AN9025S0101@adlainortye.com

14. ASP2138 (CLDN18.2xCD3 2+1 bispecific T-cell engager)

Modality: bispecific T-cell engager · **Regulatory:** Investigational; no approval · **Geographic scope:** US, Japan, EU, Asia sites; US enrollment open · **Verified:** 2026-08-12

Phase 1 recruiting with a dedicated metastatic PDAC cohort (up to 2 prior lines in expansion) and 46 sites across the US, Japan, Europe, and Asia. Requires central CLDN18.2 IHC positivity and radiographically confirmed metastatic or locally advanced unresectable disease, so both pending diagnostics feed straight into this screen. Note the monotherapy arms exclude locally advanced unresectable patients, which raises the stakes on how the recurrence gets staged.

Next steps

- 1. Call the Astellas trial line (800-888-7704, Astellas.registration@astellas.com) to ask whether the PDAC expansion is enrolling and whether archival CLDN18.2 tissue can start screening before the biopsy.
- 2. Stage the recurrence; monotherapy arms need metastatic disease.

Trial pathways

NCT	Phase	Status	Eligible	Central contact
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NCT05385581

recruiting

unconfirmed

Astellas Pharma Global Development,
Inc.;
Astellas.registration@astellas.com;
800-888-7704

Manufacturer / sponsor contact

- **Company:** Astellas Pharma
- **Med info phone:**800-727-7003
- **Med info email:**medical.communications@astellas.com
- **Product info:**<https://www.astellasanswers.com>

15. ASP546C (CLDN18.2 ADC)

Modality: ADC · **Regulatory:** Investigational; no approval · **Geographic scope:** US and Japan sites ·
Verified: 2026-08-12

Newly posted Astellas phase 1/2 recruiting at US and Japan sites. Early escalation with no published data; useful as a second US-reachable CLDN18.2 ADC while IBI343's US cohorts fill.

Next steps

1. Ask the Astellas trial line (800-888-7704) whether PDAC is enrolling and at which US sites.

Trial pathways

NCT	Phase	Status	Eligible	Central contact
NCT07488276	1/2	recruiting	unconfirmed	Astellas Pharma Global Development, Inc; Astellas.registration@astellas.com; 800-888-7704

Manufacturer / sponsor contact

- **Company:** Astellas Pharma
- **Med info phone:**800-727-7003
- **Med info email:**medical.communications@astellas.com

16. ASP5834 (IV pan-KRAS inhibitor)

Modality: small molecule · **Regulatory:** Investigational; no approval · **Geographic scope:** US, Japan, France, Spain sites · **Verified:** 2026-08-12

First-in-human trial explicitly lists KRAS G12R among eligible mutations, and its PDAC expansion counts adjuvant chemotherapy with recurrence inside 6 months as a prior line, which matches this patient's course exactly. IV administration also sidesteps post-Whipple oral-absorption questions. The constraint is that this is dose escalation,

so slots and dose levels move week to week.

Next steps

1. Call the Astellas trial line (800-888-7704, Astellas.registration@astellas.com) to ask which US sites are enrolling and whether the PDAC expansion is open at the current dose level.
2. Confirm the recurrence is documented as locally advanced unresectable or metastatic before screening.

Trial pathways

NCT	Phase	Status	Eligible	Central contact
NCT07094204		recruiting	likely	Astellas Pharma Global Development, Inc.; Astellas.registration@astellas.com ; 800-888-7704

Manufacturer / sponsor contact

- **Company:** Astellas Pharma
- **Med info phone:** 800-727-7003
- **Med info email:** medical.communications@astellas.com
- **Product info:** <https://www.astellasanswers.com>

17. AZD0901 / CMG901 (CLDN18.2 ADC, MMAE payload)

Aliases: CMG901

Modality: ADC · **Regulatory:** Investigational; no approval · **Geographic scope:** Global including US; PDAC cohort excludes this case as posted · **Verified:** 2026-08-12

The phase 2 is recruiting broadly (52 sites, US among them) and has a PDAC cohort, but that cohort is first-line and its one allowance for prior perioperative chemotherapy requires progression at least 6 months after the last dose. This patient progressed about 4 months after adjuvant FOLFIRINOX, so as posted the PDAC cohort excludes them. Captured so the treating team can ask about amendments rather than rediscover the math.

Next steps

1. One call to the AstraZeneca information center to confirm the 6-month clock is measured to the last adjuvant dose and whether any later-line PDAC cohort is planned.

Trial pathways

NCT	Phase	Status	Eligible	Central contact
NCT06229941		recruiting	no	AstraZeneca Clinical Study Information Center; information.center@astrazeneca.com ; 1-877-240-9479

Manufacturer / sponsor contact

- **Company:** AstraZeneca
- **Med info phone:** 1-877-240-9479
- **Med info email:** information.center@astrazeneca.com

18. AZD4360 (CLDN18.2 ADC)

Modality: ADC · **Regulatory:** Investigational; no approval · **Geographic scope:** US, UK, Germany, Japan, China sites (closed) · **Verified:** 2026-08-12

Phase 1/2 across US, UK, Germany, Japan, and China sites but currently closed to new enrollment, so there is no live slot today. Worth a watch given the sponsor's parallel CLDN18.2 engager (AZD5863) keeps the biology in active development.

Next steps

1. Registry watch; ask the AstraZeneca information center whether expansion cohorts will reopen enrollment.

Trial pathways

NCT	Phase	Status	Eligible	Central contact
NCT06911228	1/2	active not recruiting	no	—

Manufacturer / sponsor contact

- **Company:** AstraZeneca
- **Med info phone:** 1-877-240-9479
- **Med info email:** information.center@astrazeneca.com

19. AZD5863 (CLDN18.2xCD3 affinity-modulated T-cell engager)

Modality: bispecific T-cell engager · **Regulatory:** Investigational; no approval · **Geographic scope:** US, UK, EU, Asia sites; US enrollment open · **Verified:** 2026-08-12

Phase 1/2 recruiting with pancreatic adenocarcinoma named in the eligibility, central CLDN18.2 IHC required, and one prior line in the advanced or metastatic setting expected. The prior-line clause is the fit question for a patient whose only therapy was perioperative; everything else about the posted criteria fits once the recurrence is staged.

Next steps

1. Call 1-877-240-9479 with the treatment timeline and ask whether the prior-line requirement is satisfied.
2. Route CLDN18.2 tissue (archival now, recurrence later) to the sponsor's central lab once a site engages.

Trial pathways

NCT	Phase	Status	Eligible	Central contact
NCT06005293		recruiting	unconfirmed	AstraZeneca Clinical Study Information Center; information.center@astrazeneca.com ; 1-877-240-9479

Manufacturer / sponsor contact

- **Company:** AstraZeneca
- **Med info phone:** 1-877-240-9479
- **Med info email:** information.center@astrazeneca.com

20. BBO-11818 (pan-KRAS inhibitor)

Modality: small molecule · **Regulatory:** Investigational; no approval · **Geographic scope:** US and Australia sites · **Verified:** 2026-08-12

The registry lists eligible mutations as G12A, G12C, G12D, G12S, and G12V. G12R is not on the list, so despite the pan-KRAS label this trial does not currently take this patient. The dossier keeps it as the cautionary example that pan-KRAS programs need codon-level checking.

Next steps

1. One email to the sponsor contact asking whether G12R has been or will be added; otherwise no action.

Trial pathways

NCT	Phase	Status	Eligible	Central contact
NCT06917079		recruiting	no	TheRas, Inc., d/b/a BBOT (BridgeBio Oncology Therapeutics); tbbo11818-101ct.gov@bbotx.com ; (650) 405-4770

Manufacturer / sponsor contact

- **Company:** BBOT (BridgeBio Oncology Therapeutics / TheRas)
- **Med info phone:** (650) 405-4770
- **Med info email:** tbbo11818-101ct.gov@bbotx.com

21. BLU-924 (SAR449336, pan-KRAS inhibitor)

Aliases: SAR449336

Modality: small molecule · **Regulatory:** Investigational; no approval · **Geographic scope:** US sites (2 open) · **Verified:** 2026-08-12

Newly opened phase 1/2 at two US sites for metastatic KRAS-mutant PDAC, NSCLC, or CRC after standard therapies. Excludes prior KRAS-targeted treatment, so it competes with rather than follows daraxonrasib.

Next steps

1. Email medinfo@blueprintmedicines.com to confirm codon coverage and site openings.

Trial pathways

NCT	Phase	Status	Eligible	Central contact
NCT07621260	1/2	recruiting	unconfirmed	Blueprint Medicines; medinfo@blueprintmedicines.com ; 1-888-258-7768

Manufacturer / sponsor contact

- **Company:** Blueprint Medicines (Sanofi partnership)
- **Med info phone:** 1-888-258-7768
- **Med info email:** medinfo@blueprintmedicines.com

22. ELI-002 (lymph-node-targeted mKRAS amphiphile vaccine; 2P and 7P formulations)

Aliases: ELI-002 2P, ELI-002 7P, AMPLIFY-201, AMPLIFY-7P

Modality: vaccine · **Regulatory:** Investigational; no approval · **Geographic scope:** US sites (closed cohorts) · **Verified:** 2026-08-12

The formulation that produced the published G12R-covering immunogenicity data (2P, AMPLIFY-201) has completed, and the 7P follow-on (AMPLIFY-7P) is active but closed to new enrollment. The one recruiting trial is an MSK perioperative study in newly diagnosed resectable disease, which this patient's recurrent course does not fit. Net: the mechanism covers G12R, but there is no open door today for a recurrent patient.

Next steps

1. Watch for an Elicio trial in the overt-recurrence or metastatic setting; none is posted today.
2. The Johns Hopkins mKRASvax study (separate row) is the recruiting G12R-inclusive vaccine alternative.

Trial pathways

NCT	Phase	Status	Eligible	Central contact
NCT05711264	1/2	active not recruiting	no	—
NCT07611339	1/2	recruiting	no	Kevin Soares, MD; soaresk@mskcc.org ; 212-639-3195
NCT04833017	1/2	completed	no	—

Manufacturer / sponsor contact

- **Company:** Elicio Therapeutics
- **Notes:** No medical-information line published; Elicio posts a contact form on elicio.com.

23. IBI343 (CLDN18.2 ADC, exatecan payload)

Modality: ADC · **Regulatory:** Investigational; no approval · **Geographic scope:** Phase 1 US/Australia/China; phase 3 mainland China only · **Verified:** 2026-08-12

The largest prospective CLDN18.2 efficacy dataset in treated PDAC sits behind this drug, and its phase 1 (NCT05458219) is recruiting with US and Australia sites alongside China, so a US patient can realistically screen. The phase 3 (G-HOPE-001 successor, NCT07066098) is mainland-China only, which matters for expectations: the registrational path is running in China, and US access rides on the phase 1/2 cohorts.

Next steps

1. Email the NCT05458219 contact asking which US sites are enrolling the PDAC cohort and what CLDN18.2 expression threshold applies at 60% 2+/3+.
2. Send the CDx re-stain result with the inquiry; expression level is the gating fact for ADC cohorts.

Trial pathways

NCT	Phase	Status	Eligible	Central contact
NCT05458219	1	recruiting	likely	Suhua Dong; suhua.dong@innoventbio.com ; 0512-69566088
NCT07066098	3	recruiting	no	Penglei Zheng; penglei.zheng@innoventbio.com ; 86-512-69566088
NCT06720439	2	recruiting	no	Tingbo Liang, MD.; liangtingbo@zju.edu.cn ; 86+19941463683

Manufacturer / sponsor contact

- **Company:** Innovent Biologics
- **Med info phone:** 0512-69566088
- **Med info email:** suhua.dong@innoventbio.com
- **Notes:** Contacts are the posted trial contacts (Suzhou, China); Innovent publishes no US medical-information line.

24. JAB-23E73 (oral pan-KRAS inhibitor)

Modality: small molecule · **Regulatory:** Investigational; no approval · **Geographic scope:** US sites · **Verified:** 2026-08-12

Phase 1/2 recruiting at four US sites; eligibility asks for KRAS alterations without codon restriction as posted, so G12R plausibly fits, but that needs one confirmatory call. Prior pan-KRAS or RAS-inhibitor treatment is excluded, the same sequencing caveat as the other early pan-KRAS programs.

Next steps

1. Email clinicaltrials@jacobiopharma.com to confirm G12R fits and which US sites have slots.

Trial pathways

NCT	Phase	Status	Eligible	Central contact
NCT06912564		recruiting	unconfirmed	Jacobio Pharmaceuticals; clinicaltrials@jacobiopharma.com ; (781) 918-6670

Manufacturer / sponsor contact

- **Company:** Jacobio Pharmaceuticals
- **Med info phone:**(781) 918-6670
- **Med info email:**clinicaltrials@jacobiopharma.com

25. JAB-30355 (oral p53 Y220C reactivator)

Modality: small molecule · **Regulatory:** Investigational; no approval · **Geographic scope:** US and China sites; US enrollment open · **Verified:** 2026-08-12

The practical Y220C route for this KRAS-co-mutated patient: phase 1/2 recruiting at US sites, requires one prior line for the tumor type and documented Y220C, and posts no KRAS exclusion. The efficacy record is essentially a single conference-level PDAC response, so this is an eligibility fact more than an evidence bet, and the contacts below let a clinician probe directly.

Next steps

1. Email clinicaltrials@jacobiopharma.com with the Y220C documentation and treatment history; ask about US site slots and whether KRAS co-mutation affects cohort assignment.
2. Confirm Y220C persistence in the recurrence via ctDNA or biopsy before dosing paperwork starts.

Trial pathways

NCT	Phase	Status	Eligible	Central contact
NCT06386246		recruiting	likely	Jacobio Pharmaceuticals; clinicaltrials@jacobiopharma.com ; 781-918-6670

Manufacturer / sponsor contact

- **Company:** Jacobio Pharmaceuticals

- **Med info phone:**781-918-6670
- **Med info email:**clinicaltrials@jacobiopharma.com

26. KST-6051 (oral pan-KRAS inhibitor)

Modality: small molecule · **Regulatory:** Investigational; no approval · **Geographic scope:** US sites · **Verified:** 2026-08-12

Phase 1 first-in-human, US sites, and eligibility as posted asks only for a documented KRAS mutation, which G12R satisfies on its face. No prior RAS-inhibitor exposure is allowed, so sequencing matters: this trial is unreachable after daraxonrasib.

Next steps

1. Email clinicaltrials@kestreltherapeutics.com to confirm G12R enrollment and site openings.
2. If daraxonrasib is the chosen first move, note this trial's prior-RAS-inhibitor exclusion when planning sequence.

Trial pathways

NCT	Phase	Status	Eligible	Central contact
NCT07438347		recruiting	likely	Kestrel Therapeutics, Inc.; clinicaltrials@kestreltherapeutics.com ; 617-612-6810

Manufacturer / sponsor contact

- **Company:** Kestrel Therapeutics
- **Med info phone:**617-612-6810
- **Med info email:**clinicaltrials@kestreltherapeutics.com

27. LB1908 (CLDN18.2 CAR-T)

Modality: CAR-T · **Regulatory:** Investigational; no approval · **Geographic scope:** US sites (closed) · **Verified:** 2026-08-12

US phase 1 is active but closed to new enrollment and posts no central contact, so there is no door today. Registry watch only.

Next steps

1. Watch [NCT05539430](#) for reopening or a phase 2 posting.

Trial pathways

NCT	Phase	Status	Eligible	Central contact
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Manufacturer / sponsor contact

- **Company:** Legend Biotech

28. LG00313112 (oral p53 Y220C reactivator)

Modality: small molecule · **Regulatory:** Investigational; no approval · **Geographic scope:** Not yet determined (no sites posted) · **Verified:** 2026-08-12

Posted but not yet recruiting, with no sites listed and no central contact. Nothing to call yet; worth a registry watch given how thin the Y220C field is.

Next steps

1. Set a registry alert on NCT07752875 and re-check monthly.

Trial pathways

NCT	Phase	Status	Eligible	Central contact
NCT07752875		not yet recruiting	unconfirmed	—

Manufacturer / sponsor contact

- **Company:** LG Chem

29. Mutant-KRAS TCR-T programs (NT-112/AZD0240, Anocca ANOC-001/2/3, TCR1188)

Aliases: NT-112, AZD0240, ANOC-001, ANOC-002, ANOC-003, TCR1188-ABC

Modality: TCR-T · **Regulatory:** Investigational; no approval · **Geographic scope:** US and EU sites, none applicable to G12R · **Verified:** 2026-08-12

Every recruiting mutant-KRAS TCR-T trial on the registry restricts to G12D or G12V, so none of them takes a G12R tumor; this row documents the coverage gap rather than a path. The contacts are captured so a clinician can ask each sponsor directly whether a G12R construct is coming.

Next steps

1. One email to clinical.trials@anocca.com asking whether a G12R-restricted TCR is in their library plans; their platform is built to broaden allele coverage.
2. No enrollment action otherwise; G12R is not covered by any open TCR-T trial.

Trial pathways

NCT	Phase	Status	Eligible	Central contact
NCT06218914		recruiting	no	AstraZeneca Clinical Study Information Center; information.center@astrazeneca.com; 1-877-240-9479
NCT07147250		recruiting	no	Amu Wang; amu.wang@anocca.com; +46703784946
NCT07594067		recruiting	no	Abramson Cancer Center Clinical Trials Service; 215-349-8245

Manufacturer / sponsor contact

- **Company:** AstraZeneca (NT-112); Anocca AB; University of Pennsylvania
- **Med info phone:** 1-877-240-9479
- **Med info email:** information.center@astrazeneca.com
- **Notes:** Phone/email are the AstraZeneca study information center from the NT-112 registry record. Anocca trial contact: clinical.trials@anocca.com, +46841080701.

30. NTS071 (oral p53 Y220C reactivator)

Modality: small molecule · **Regulatory:** Investigational; no approval · **Geographic scope:** US and China sites; US enrollment open · **Verified:** 2026-08-12

Second US-recruiting Y220C option without a posted KRAS exclusion; requires one or more prior lines and progression, with prior-therapy toxicity resolved to grade 1 or baseline, which makes the neuropathy grading row directly relevant. Evidence base is essentially empty so far, so treat this as a door to keep open rather than a ranked bet.

Next steps

1. Email the posted contact with Y220C documentation and ask which US site is enrolling and at what dose level.
2. Document the oxaliplatin neuropathy grade; residual grade 2 would conflict with the toxicity-resolution criterion.

Trial pathways

NCT	Phase	Status	Eligible	Central contact
NCT07081289		recruiting	likely	Yan Xu; yan.xu@allonutshell.com ; +86 13585878226

Manufacturer / sponsor contact

- **Company:** Nutshell Therapeutics
- **Med info email:** yan.xu@allonutshell.com
- **Notes:** Email is the posted trial contact; no separate US medical-information line published.

31. PF-07934040 (pan-KRAS inhibitor)

Modality: small molecule · **Regulatory:** Investigational; no approval · **Geographic scope:** US and Puerto Rico sites · **Verified:** 2026-08-12

Investigational with a single phase 1 that is active but not recruiting, so there is nothing to enroll in right now. No central contact is posted on the registry record.

Next steps

1. Watch NCT06447662 for a status change; no live path today.

Trial pathways

NCT	Phase	Status	Eligible	Central contact
NCT06447662		active not recruiting	no	—

Manufacturer / sponsor contact

- **Company:** Pfizer
- **Notes:** No central contact posted on the registry record; route questions through the treating center's Pfizer study liaison if one exists.

32. QLS31905 / ANO31905 (CLDN18.2xCD3 bispecific) + gemcitabine/nab-paclitaxel

Aliases: ANO31905

Modality: bispecific T-cell engager · **Regulatory:** Investigational; no approval · **Geographic scope:** One US site (San Antonio); phase 3 mainland China only · **Verified:** 2026-08-12

Two doors, one reachable: the US phase 1/2 (Anova, NCT07444541) is recruiting at the Mays Cancer Center in San Antonio with a named investigator contact, while the phase 3 runs in mainland China only. For a patient willing to travel domestically, the San Antonio study is a concrete CLDN18.2 engager option in first-line-style combination with chemotherapy.

Next steps

1. Email the posted investigator contact (zhengl@uthscsa.edu, 210-450-5798) with the case summary and CLDN18.2 report.

Trial pathways

NCT	Phase	Status	Eligible	Central contact
NCT07444541		recruiting	unconfirmed	Lei Zheng Mays Cancer Center; zhengl@uthscsa.edu; 210-450-5798
NCT07039228		recruiting	no	Lin Shen, M.D; linshenpku@163.com; 010-88196561

Manufacturer / sponsor contact

- **Company:** Anova Innovation (US development); Qilu Pharmaceutical (originator)

33. TAC01-CLDN18.2 (T-cell antigen coupler cell therapy)

Modality: engineered T cell · **Regulatory:** Investigational; no approval · **Geographic scope:** US and Canada sites (closed) · **Verified:** 2026-08-12

US and Canada phase 1/2, currently closed to new enrollment. Its criteria are notable for a clause enrolling incurable CLDN18.2-positive cancers regardless of prior-line count when no targeted standard exists, which would have suited this case; that makes it worth the reopening watch.

Next steps

1. Watch for reopening; screen the recruiting cell-therapy alternatives meanwhile.

Trial pathways

NCT	Phase	Status	Eligible	Central contact
NCT05861224		active not recruiting	no	—

Manufacturer / sponsor contact

- **Company:** Triumvira Immunologics

34. TG01 mutant-KRAS peptide vaccine + daratumumab + nivolumab (Georgetown)

Modality: vaccine · **Regulatory:** Investigational; no approval · **Geographic scope:** Single US site (closed) · **Verified:** 2026-08-12

The Georgetown phase 2 is active but closed to new enrollment, and no other US TG01 trial is recruiting, so there is no live door. Listed to keep the vaccine landscape complete.

Next steps

1. No action; the Hopkins mKRASvax study is the recruiting alternative in this class.

Trial pathways

NCT	Phase	Status	Eligible	Central contact
NCT06025724		active not recruiting	no	—

35. TNhYP218 (mesothelin-targeted CAR-T, NCI)

Modality: CAR-T · **Regulatory:** Investigational; no approval · **Geographic scope:** Single US site (Bethesda, MD); NCI covers some travel · **Verified:** 2026-08-12

NCI intramural phase 1, recruiting at the Bethesda Clinical Center with named, reachable contacts. Mesothelin expression on the tumor has not been assessed in this case, so screening starts with an expression question rather than a slot question. NCI trials cover travel for enrolled patients, which fits this household's stated willingness to travel.

Next steps

1. Email the posted referral contact (mariagracia.agra@nih.gov, (240) 858-3152) with records; NCI screening will handle mesothelin testing.
2. Ask about the NIH travel-support policy at first contact.

Trial pathways

NCT	Phase	Status	Eligible	Central contact
NCT06885697		recruiting	unconfirmed	Maria Gracia L Agra, R.N.; mariagracia.agra@nih.gov; (240) 858-3152

Manufacturer / sponsor contact

- **Company:** National Cancer Institute (sponsor-investigator)

36. TORL-2-307-ADC (CLDN18.2 ADC)

Modality: ADC · **Regulatory:** Investigational; no approval · **Geographic scope:** US and South Korea sites (closed) · **Verified:** 2026-08-12

Phase 1 at US and South Korea sites, closed to new enrollment, no central contact posted. No live path; registry watch only.

Next steps

1. Watch NCT05156866 for expansion-phase reopening.

Trial pathways

NCT	Phase	Status	Eligible	Central contact
NCT05156866		active not recruiting	no	—

Manufacturer / sponsor contact

- **Company:** TORL Biotherapeutics

37. YL-17231 (pan-RAS inhibitor)

Modality: small molecule · **Regulatory:** Investigational; no approval · **Geographic scope:** US sites · **Verified:** 2026-08-12

Phase 1 recruiting at three US sites (the posted central contact is at MD Anderson) for RAS-mutant solid tumors after standard therapy. Early escalation with no published efficacy, so this is a backup conversation rather than a first call.

Next steps

1. Contact the posted trial contact (David S. Hong, MD, MD Anderson) about slots and dose level.

Trial pathways

NCT	Phase	Status	Eligible	Central contact
NCT06096974		recruiting	likely	David S. Hong, MD; dshong@mdanderson.org; (713) 563-1930

Manufacturer / sponsor contact

- **Company:** Shanghai YingLi Pharmaceutical
- **Notes:** No US medical-information line published; the trial contact at MD Anderson is the practical door.

38. daraxonrasib (RMC-6236, pan-RAS(ON) multi-selective inhibitor)

Aliases: RMC-6236, RMC-0706236

Modality: small molecule · **Regulatory:** Investigational; NDA accepted for FDA review July 2026 (previously treated metastatic PDAC), Breakthrough Therapy and Orphan Drug designations, FDA national priority voucher; FDA-authorized expanded access protocol active since May 2026 · **Geographic scope:** EAP is US-only (physician-submitted); RASolute 303 sites in US, Japan, Australia; phase 1/2 US-only · **Verified:** 2026-08-12

Two live routes, and the expanded-access one is real: FDA authorized Revolution Medicines' EAP (protocol RMC-6236-401, NCT07573215) in May 2026 for previously treated metastatic PDAC, with dozens of US sites listed as available and drug supplied free of charge; the NDA was accepted for FDA review in July 2026. RASolute 302 is closed to enrollment, but the first-line phase 3 (RASolute 303) and the phase 1/2 remain open. The load-bearing constraint for this patient is staging: both the EAP and 303 require confirmed metastatic disease, and 303 additionally requires consent within 6 weeks of the metastatic diagnosis, so the outreach call has to be teed up before the biopsy result lands, not after.

Next steps

1. Before the biopsy result returns, have the oncologist identify the nearest RASolute 303 site (site list on NCT07491445) and pre-alert them, so consent can happen inside the 6-week window if staging reads metastatic.
2. If 303 does not fit (locally recurrent rather than metastatic disease, slot unavailability, or the team prefers

monotherapy), the physician emails medinfo@revmed.com to open an EAP request under NCT07573215.

3. Ask the same contact whether the phase 1/2 (NCT05379985) or a G12R-accepting RASolve 101 arm (NCT06445062) has nearer-term slots.
4. Clarify with the sponsor whether adjuvant FOLFIRINOX with recurrence at 4 months satisfies the EAP's prior-line requirement.

Trial pathways

NCT	Phase	Status	Eligible	Central contact
NCT07573215		—	unconfirmed	—
NCT07431445		recruiting	likely	Revolution Medicines Study Director; medinfo@revmed.com ; 1-844-2-REVMED
NCT05379985		recruiting	likely	Revolution Medicines, Inc.; medinfo@revmed.com ; 1-844-273-8633
NCT06445062		recruiting	unconfirmed	Revolution Medicines; medinfo@revmed.com ; 1-844-2-REVMED
NCT06625320		active not recruiting	no	—

Manufacturer / sponsor contact

- **Company:** Revolution Medicines
- **Med info phone:** 1-844-2-REVMED
- **Med info email:** medinfo@revmed.com
- **Product info:** <https://www.revmed.com>
- **Compassionate / expanded access:**
<https://ir.revmed.com/news-releases/news-release-details/revolution-medicines-statement-fda-expanded-access-authorization>
- **Notes:** EAP requests must come from a licensed treating physician; the company does not accept patient or caregiver requests directly. Phone and email are verbatim from the sponsor's registry records.

Payer / coverage: Drug is supplied free under the EAP. Press reporting in June 2026 (STAT) described demand outstripping the program's early capacity, so expect a queue and ask the sponsor about realistic start timelines.

Notes: The highest-value access finding in this case: an approved-track drug with a formal, currently available expanded-access protocol in exactly this disease.

39. givastomig (TJ033721/ABL111, CLDN18.2x4-1BB bispecific)

Aliases: TJ033721, ABL111

Modality: bispecific antibody · **Regulatory:** Investigational; no approval · **Geographic scope:** US and China sites; US enrollment open · **Verified:** 2026-08-12

Phase 1 recruiting at US and China sites, with a pancreatic combination arm and a mechanism that tolerates a

wide expression range, relevant to a 60% tumor. Prior anti-PD-1 exposure is excluded in some combination parts, which this patient satisfies, having had none.

Next steps

1. Email us.info@imabbio.com asking whether the PDAC combination cohort is open at a US site and what expression cutoff applies.

Trial pathways

NCT	Phase	Status	Eligible	Central contact
NCT04900818		recruiting	unconfirmed	Clinical Development; us.info@imabbio.com ; (240) 745-6330

Manufacturer / sponsor contact

- **Company:** I-Mab Biopharma / ABL Bio
- **Med info phone:**(240) 745-6330
- **Med info email:**us.info@imabbio.com

40. mKRASvax pooled mutant-KRAS peptide vaccine + balstilimab + botensilimab (Johns Hopkins)

Modality: vaccine · **Regulatory:** Investigational; no approval · **Geographic scope:** Single US site (Baltimore, MD) · **Verified:** 2026-08-12

Investigator-initiated phase 1/2 at Johns Hopkins, recruiting, and the vaccine pool includes G12R. The fit question is the prior-therapy clause: the PDAC cohort as posted wants 4 to 6 months of first-line FOLFIRINOX or gemcitabine/nab-paclitaxel for metastatic disease, and this patient's 10 cycles were neoadjuvant and adjuvant. That reads as a coordinator conversation, not a hard stop.

Next steps

1. Email GIClinicalTrials@jhmi.edu with the treatment timeline and ask whether perioperative FOLFIRINOX with early recurrence satisfies the cohort's prior-therapy requirement.
2. Flag the on-treatment biopsy requirement against the vascular-invasion anatomy when the EUS team weighs in.

Trial pathways

NCT	Phase	Status	Eligible	Central contact
NCT06411691		recruiting	unconfirmed	Colleen Apostol, RN; GIClinicalTrials@jhmi.edu ; 410-614-3644

41. rezatapopt (PC14586, p53 Y220C reactivator)

Aliases: PC14586

Modality: small molecule · **Regulatory:** Investigational; no approval (PYNNACLE registrational program ongoing) · **Geographic scope:** US, EU, Australia, Asia sites for PYNNACLE; expanded access via US physicians · **Verified:** 2026-08-12

The registrational PYNNACLE phase 2 monotherapy cohort excludes tumors with a KRAS single-nucleotide variant, which this G12R tumor has, so the flagship Y220C program is closed to this patient as posted. PMV does run a case-by-case expanded-access process for patients who cannot join a trial and have exhausted options, initiated by the treating physician by email. The practical recruiting Y220C routes for this patient are JAB-30355 and NTS071 (separate rows).

Next steps

1. Confirm Y220C persists in the recurrence (ctDNA suffices) before spending calls on this class.
2. Have the physician email clinicaltrials@pmvpharma.com to ask whether any PYNNACLE arm or the expanded-access process can take a KRAS-co-mutated patient.
3. Screen JAB-30355 (NCT06386146) and NTS071 (NCT07060989) in parallel; neither posts a KRAS exclusion.

Trial pathways

NCT	Phase	Status	Eligible	Central contact
NCT04581250	1/2	recruiting	no	PMV Pharma Clinical Study Information Center; clinicaltrials@pmvpharma.com ; (609) 235-4038

Manufacturer / sponsor contact

- **Company:** PMV Pharmaceuticals
- **Med info phone:**(609) 235-4038
- **Med info email:**clinicaltrials@pmvpharma.com
- **Product info:**<https://www.pmvpharma.com/pipeline/>
- **Compassionate / expanded access:**<https://www.pmvpharma.com/pipeline/access/>
- **Compassionate use email:**clinicaltrials@pmvpharma.com

42. sacituzumab tirumotecan (sac-TMT, MK-2870/SKB264, TROP2 ADC)

Aliases: MK-2870, SKB264

Modality: ADC · **Regulatory:** NMPA-approved (2L+ TNBC, China); investigational in the US, FDA breakthrough designation in EGFR-mutant NSCLC · **Geographic scope:** US sites among 13 countries · **Verified:** 2026-08-12

Approved in China for later-line triple-negative breast cancer but investigational in the US, where the door for this case is Merck's phase 1/2 basket with a previously treated PDAC arm, recruiting at US sites among 13 countries. TROP2 is broadly expressed in PDAC and the arm requires prior therapy, which the perioperative FOLFIRINOX

plausibly satisfies; confirm with the sponsor.

Next steps

1. Call 1-888-577-8839 or email Trialsites@msd.com to locate the nearest US site with PDAC-arm slots and confirm the prior-therapy definition.

Trial pathways

NCT	Phase	Status	Eligible	Central contact
NCT06412209	1/2	recruiting	unconfirmed	Toll Free Number; Trialsites@msd.com ; 1-888-577-8839

Manufacturer / sponsor contact

- **Company:** Merck (ex-China rights; Kelun-Biotech originator)
- **Med info phone:** 1-800-672-6372
- **Med info email:** Trialsites@msd.com
- **Notes:** Email is the posted trial contact; phone is the Merck National Service Center.

43. spevatamig (PT886, CLDN18.2xCD47 bispecific)

Aliases: PT886

Modality: bispecific antibody · **Regulatory:** Investigational; no approval (FDA orphan and fast track designations reported for pancreatic cancer) · **Geographic scope:** US sites · **Verified:** 2026-08-12

Phase 1/2 recruiting at 11 US sites for previously treated advanced gastric, GEJ, biliary, and pancreatic carcinomas, with tissue submission for CLDN18.2 assessment built into screening. A straightforward US door in the CLDN18.2 class once the recurrence is documented.

Next steps

1. Email clinical-trials@phanestx.com for site openings and whether archival tissue can pre-screen while the biopsy is scheduled.

Trial pathways

NCT	Phase	Status	Eligible	Central contact
NCT05481293	1/2	recruiting	unconfirmed	Phanes Therapeutics; clinical-trials@phanestx.com ; 858-766-0852

Manufacturer / sponsor contact

- **Company:** Phanes Therapeutics
- **Med info phone:** 858-766-0852

- **Med info email:**clinical-trials@phanestx.com

44. zoldonrasib (RMC-9805, KRAS G12D(ON)-selective inhibitor)

Aliases: RMC-9805

Modality: small molecule · **Regulatory:** Investigational; no approval · **Geographic scope:** US sites · **Verified:** 2026-08-12

Investigational and G12D-selective, so its open phase 3 does not read on a G12R tumor. Kept on the ledger to document the allele gate rather than as a usable route.

Next steps

1. No action; the allele does not match. Daraxonrasib is the Revolution Medicines route for G12R.

Trial pathways

NCT	Phase	Status	Eligible	Central contact
NCT07621718		recruiting	no	Revolution Medicines Study Director; medinfo@revmed.com ; 1-844-2-REVMED

Manufacturer / sponsor contact

- **Company:** Revolution Medicines
- **Med info phone:** 1-844-2-REVMED
- **Med info email:** medinfo@revmed.com
- **Product info:** <https://www.revmed.com>

Not yet accessible (8)

45. 177Lu-DOTA-SNA040 (CLDN18.2-directed lutetium-177 radioligand therapy)

Aliases: SNA040, [177Lu]Lu-DOTA-SNA040, 68Ga-NODAGA-SNA014 (imaging companion)

Modality: radioligand · **Regulatory:** Investigational; no approval anywhere, no US IND posted · **Geographic scope:** Mainland China only; single center (Shanghai) · **Verified:** 2026-08-12

The biomarker fit is the best in the whole CLDN18.2 file, and the honest verdict is still that a US patient cannot get this drug. The only trial is a 12-patient investigator-initiated dose escalation at a single Shanghai center (Fudan University Shanghai Cancer Center), with 2 to 6 treatment cycles after a 68Ga-PET gate, meaning months of care inside the Chinese health system. Single-center Chinese IITs at this scale do not have a mechanism for enrolling foreign patients: consent, insurance, safety follow-up, and China's human-genetic-resources rules all assume local residents, and nothing on the record suggests the sponsor has built an exception. Stated travel willingness does

not bridge that; this is the satri-cel situation one stage earlier, minus the approval.

Next steps

- 1. One email to mengru.wu@smartnucl.com from the treating team, asking two questions: does the IIT have any mechanism for a non-resident patient, and is a US or multinational sponsor study planned. Expect the first answer to be no; the second is the useful one.
- 2. Set a registry alert for new SNA040 or SmartNuclide postings; a US IND would change this row's verdict outright.
- 3. Keep the 43-14A CDx re-stain report on file; if this class ever opens in the West, this tumor walks in on biomarker.

Trial pathways

NCT	Phase	Status	Eligible	Central contact
NCT07562197	early	recruiting	likely	shaoli song, doctor; mengru.wu@smartnucl.com; 0512-67229125

Manufacturer / sponsor contact

- **Company:** SmartNuclide Biopharma (Suzhou, China)
- **Med info phone:**0512-67229125
- **Med info email:**mengru.wu@smartnucl.com
- **Notes:** Contacts are the posted trial contacts (Suzhou office); no English-language medical-information line or expanded-access page found. No US or ex-China development posted anywhere on the registry.

Notes: Decision-relevant class finding: a live registry query this run found exactly three CLDN18.2 radioconjugate studies worldwide (this one, IR199 at Huashan, and an imaging-only immunoPET study NCT06602037), all in China. No Western company has a CLDN18.2 radioligand in the clinic or a posted plan for one; the closest Western-visible work is academic preclinical (a 131I-zolbetuximab mouse study). So the modality best matched to this tumor's expression pattern currently has no reachable door at all.

46. AZD6422 (TGF-beta-armored CLDN18.2 CAR-T; class exemplar for stromal suppression)

Aliases: AZD6422

Modality: CAR-T · **Regulatory:** Investigational; no approval · **Geographic scope:** No open trial anywhere · **Verified:** 2026-08-12

The armored-CAR-T exemplar the researcher track flagged has completed its only posted trial, a phase 1 in China, and no US or follow-on study is on the registry. The class idea (engineering around TGF-beta suppression in the PDAC stroma) currently has no enrollable vehicle; the reachable CLDN18.2 cell-therapy doors are the unarmored programs listed in their own rows.

Next steps

1. Ask the AstraZeneca information center whether a global AZD6422 study is planned; otherwise no action.

Trial pathways

NCT	Phase	Status	Eligible	Central contact
NCT05984235		—	no	—

Manufacturer / sponsor contact

- **Company:** AstraZeneca
- **Med info phone:** 1-877-240-9479
- **Med info email:** information.center@astrazeneca.com

47. FG-M108 (ADCC-enhanced anti-CLDN18.2 mAb) + gemcitabine/nab-paclitaxel

Aliases: M108

Modality: monoclonal antibody · **Regulatory:** Investigational; no approval · **Geographic scope:** Mainland China only, not yet recruiting · **Verified:** 2026-08-12

The first PDAC phase 3 in the CLDN18.2 class whose cutoff (40% or more at 2+ or higher) this tumor actually clears at 60%, which is why it matters, and the reason its inaccessibility stings: the trial is posted at a single mainland-China site and is not yet recruiting. No US IND or site is posted. The right move is a watch, plus one email to the sponsor about global expansion plans.

Next steps

1. Email pr@futuregenbiopharm.com asking whether the phase 3 will open ex-China sites or whether any global study is planned.
2. Keep the CDx re-stain report ready; at 60% this is one of the few programs where the number is an asset rather than a caveat.

Trial pathways

NCT	Phase	Status	Eligible	Central contact
NCT07383922		not yet recruiting	unconfirmed	Zhaoyu Jin, Ph.D; pr@futuregenbiopharm.com ; 010-60709130

Manufacturer / sponsor contact

- **Company:** FutureGen Biopharmaceutical
- **Med info phone:** 010-60709130
- **Med info email:** pr@futuregenbiopharm.com
- **Notes:** Contact is the posted trial contact (Beijing).

48. GenSci128 (p53 Y220C reactivator)

Modality: small molecule · **Regulatory:** Investigational; no approval · **Geographic scope:** Mainland China only · **Verified:** 2026-08-12

Phase 1 recruiting at a single mainland-China site with no US IND posted. Not a realistic route for a US patient even with travel; the US-recruiting Y220C options are JAB-30355 and NTS071.

Next steps

1. No US action; revisit if a US site appears on the registry.

Trial pathways

NCT	Phase	Status	Eligible	Central contact
NCT06908434		recruiting	unconfirmed	Wen Xu; xuwen01@genscigroup.com; +8618861095711

49. JYP0015 (ERAS-0015, pan-RAS(ON) molecular glue)

Aliases: ERAS-0015

Modality: small molecule · **Regulatory:** Investigational; no approval · **Geographic scope:** Mainland China only · **Verified:** 2026-08-12

The posted phase 2 runs at a single mainland-China site, and no US trial of this molecule is on the registry. Even with travel on the table, enrolling a US patient into a China-only site is not a realistic route; sponsors generally require local care continuity.

Next steps

1. No US action; revisit if Erasca or the sponsor opens a US IND for this molecule.

Trial pathways

NCT	Phase	Status	Eligible	Central contact
NCT06825031		recruiting	unconfirmed	Ling Shen, M.D.; linshenpku@163.com; 01088121122

50. XNW27011 (CLDN18.2 ADC, licensed by Astellas from Evopoint)

Modality: ADC · **Regulatory:** Investigational; no approval · **Geographic scope:** Mainland China only, closed · **Verified:** 2026-08-12

The posted phase 1/2 runs at a single mainland-China site and is closed to new enrollment. Astellas' license suggests global development may follow, but nothing reachable is posted today.

Next steps

1. Watch for an Astellas global study posting; ask their medical line if one is imminent.

Trial pathways

NCT	Phase	Status	Eligible	Central contact
NCT06791235	1/2	active not recruiting	no	—

Manufacturer / sponsor contact

- **Company:** Astellas Pharma (license) / Evopoint Biosciences
- **Med info phone:** 800-727-7003
- **Med info email:** medical.communications@astellas.com

51. [68Ga]/[131I]-IR199 (CLDN18.2-directed theranostic pair, single dose)

Aliases: IR199, [68Ga]-IR199, [131I]-IR199

Modality: radioligand · **Regulatory:** Investigational; no approval anywhere, no US IND posted · **Geographic scope:** Mainland China only; single center (Shanghai) · **Verified:** 2026-08-12

Not a route for this patient, twice over. First, the same reachability wall as SNA040: a 10-patient investigator-initiated study at one Shanghai hospital (Huashan), with no mechanism for foreign enrollment. Second, even for a local patient this is not treatment: the primary endpoint is biodistribution and dosimetry from a single administration, so it answers an imaging-physics question rather than offering a therapy course. It matters to this dossier as proof the CLDN18.2 radioconjugate class has a second independent clinical program, and for nothing else yet.

Next steps

1. No outreach worth the patient's time; a single-dose dosimetry study in Shanghai offers neither access nor treatment intent for a US patient.
2. Watch for a follow-on therapeutic IR199 study; a dosimetry readout usually precedes one, and a multi-center successor would be the record to re-screen.

Trial pathways

NCT	Phase	Status	Eligible	Central contact
NCT07704911	early	recruiting	unconfirmed	Yihui Guan, MD; guanyihui@hotmail.com ; +86 13764308300

Manufacturer / sponsor contact

- **Company:** Huashan Hospital (investigator-initiated; Prof. Yihui Guan, responsible party)
- **Med info phone:**+86 13764308300
- **Med info email:**guanyihui@hotmail.com
- **Notes:** Academic IIT; contacts are the posted investigator contacts, and no company partner is named on the record.

52. satri-cel (CT041, autologous CLDN18.2 CAR-T)

Aliases: CT041, satricabtagene autoleucel

Modality: CAR-T · **Regulatory:** NMPA-approved June 2026 (CLDN18.2+ gastric/GEJ, 2L+); investigational in the US, no FDA filing announced · **Geographic scope:** Approved and dispensed in mainland China only; North American trial closed · **Verified:** 2026-08-12

Approved, but not anywhere this patient can reach: China's NMPA approved satri-cel in June 2026 for CLDN18.2-positive gastric/GEJ cancer after two prior lines, the first CAR-T approval in a solid tumor. The North American phase 1b/2 (ELIMYN18.2) is active but closed to new enrollment, there is no US approval or expanded-access posting, and CARsgen has not announced a named-patient route outside China. Treatment in China would mean self-funded care wholly outside US programs, on a gastric label this PDAC case does not match anyway.

Next steps

1. Have the treating team email PR@carsgen.com asking about US reopening, expanded-access plans, or any named-patient route for pancreatic cancer.
2. Watch [NCT04404595](#) for a status change or new cohorts.
3. Screen the reachable CLDN18.2 cell and engager programs (TAC01, AZD5863, ASP2138 rows) in the meantime.

Trial pathways

NCT	Phase	Status	Eligible	Central contact
NCT04404595		active not recruiting	no	—
NCT04581273		active not recruiting	no	—

Manufacturer / sponsor contact

- **Company:** CARsgen Therapeutics
- **Med info email:**PR@carsgen.com
- **Product info:**<https://www.carsgen.com/en/>
- **Notes:** No English-language medical-information line published; PR@carsgen.com and IR@carsgen.com are the posted contacts. CEO statements mention expanding to more countries without timelines.

Unavailable (1)

53. EO-3021 / SYSA1801 (CLDN18.2 ADC, MMAE payload)

Aliases: SYSA1801

Modality: ADC · **Regulatory:** Discontinued (Elevation Oncology, March 2025); no approval · **Geographic scope:** None · **Verified:** 2026-08-12

Discontinued. Elevation Oncology stopped EO-3021 development in March 2025 after a 22% response rate in gastric cancer that the company judged uncompetitive in this class, and the phase 1 was terminated. No successor program or access route exists in the US; CSPC's parallel SYSA1801 development has remained China-based. The answer to 'can we get it' is no.

Next steps

- 1. None; the recruiting CLDN18.2 ADC alternatives are IBI343 and ASP546C.

Trial pathways

NCT	Phase	Status	Eligible	Central contact
NCT05980416		terminated	no	—

Manufacturer / sponsor contact

- **Company:** Elevation Oncology (US rights); CSPC Pharmaceutical (originator)
- **Notes:** Elevation pivoted to EO-1022 and reduced its workforce; no EO-3021 access program exists.